

## **ADMISSION NOTE**

Date of Admission: 15/04/2026

Time: 10:30 AM

Admitting Doctor: Dr. [unclear]

Ward: General Medicine - Room 204

### **PATIENT DETAILS:**

Name: [NOT DOCUMENTED]

Age/Gender: 58 years / Male

IP Number: [NOT DOCUMENTED]

Blood Group: B+

### **CHIEF COMPLAINTS:**

- Severe headache since 2 days
- Blurring of vision since 1 day
- Nausea and vomiting x 3 episodes
- No chest pain, no shortness of breath

### **HISTORY OF PRESENT ILLNESS:**

Patient is a known case of hypertension on irregular medication. Presented with sudden onset severe headache, blurring of vision and nausea. No history of similar episodes in the past. Patient was brought by family members.

### **PAST HISTORY:**

- Known case of Hypertension - on Tab. Amlodipine 5mg OD
- Known case of Type 2 Diabetes Mellitus - on Tab. Metformin 500mg BD
- No known drug allergies - PENICILLIN ALLERGY DOCUMENTED (conflicting entry - requires clarification)

### **ALLERGIC HISTORY:**

Penicillin - Rash

### **PHYSICAL EXAMINATION:**

General: Conscious, oriented, anxious

PR: 92 bpm

BP: 210/120 mmHg (RIGHT ARM)

RR: 18/min

SP02: 97% on room air

Temperature: 98.6 F

### **GCS: 15/15**

CNS: Conscious and oriented. No focal neurological deficit.

CVS: S1 S2 heard. No murmurs.

RS: B/L air entry equal. No added sounds.

PA: Soft, non-tender. No organomegaly.

FUNDOSCOPY: Papilloedema present bilaterally.

### **PROVISIONAL DIAGNOSIS:**

Hypertensive Emergency with Hypertensive Retinopathy

### **INVESTIGATIONS ORDERED:**

- CBC
- Renal Function Test (RFT) - REPORT AWAITED

- Serum Electrolytes
- **ECG**
- CT Brain - Plain
- Urine Routine
- Fundoscopy

**MANAGEMENT PLAN:**

- IV access secured
- Tab. Amlodipine 5mg OD continued
- Inj. Labetalol 20mg IV given
- Inj. Furosemide 40mg IV given
- BP monitoring every 30 minutes
- Strict input/output monitoring
- Low salt diet advised

Resident Doctor Signature: [unclear]

Consultant Signature: [unclear]